

Medical
M-06 Poisoning / Overdose BLS

Revised 3/1/2000

PRIORITIES:

- Approach patient(s) after assessing appropriate safety for personnel
- ABCs
- Airway maintenance
- Determine type, amount and length of time of exposure by the patient
- Bring in the container and/or label
- Initiate treatment under the direction of the base hospital or poison control center.

Toxic Injections and Exposures (Basic Therapy)

1. Ensure a patent airway
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY – Begin oxygen at 6 liters/ minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respirations as needed. **DO NOT** withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD.
4. Position of comfort if conscious. If depressed level of consciousness, position on left side;;
5. Assist advanced life support personnel with patient packaging and movement to ambulance;
6. Consider:
 - Contact Base Hospital or Poison Control Center;
 - Being careful not to contaminate yourself and others, remove contaminated clothing, brush off powders, wash off liquids.
 - If patient is fully conscious and alert, do not attempt to dilute injected substances with water or milk until an ALS unit arrives or landline consultation with a base hospital/poison control center;
 - Skin contact with toxic agent: Remove patient from contact with source. Remove clothing from patient if contaminated. If the contact substance is a powder, brush off first, then wash off, otherwise wash copiously with water as indicated (up to 20 mins., or more may be indicated).
 - Locate containers or types of substance, for transport to hospital for identification (if safe to handle). If MSDS printouts are available bring them to the hospital with the patient.
 - Monitor vital signs and level of consciousness. Watch for seizures;
 - Observe patient at all times. Never leave a patient alone.

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PRIORITIES:

- Approach patients after assessing appropriate safety for personnel.
- ABCs.
- Determine type, amount, time of material absorbed by patient.
- Being careful not to contaminate yourself and others, remove contaminated clothing, brush off powders, wash off liquids.
- Bring in the container and/or label.
- Early transport.
- **EARLY CONTACT OF RECEIVING HOSPITAL.**

**Effective
6/15/12019**

**Review
6/15/2021**

PATIENT TREATMENT GUIDELINES

- Stabilize airway;
- **OXYGEN Therapy** – use appropriate adjuncts;
- Cardiac monitor;
- IV Access TKO, if hypotensive, give 250cc fluid bolus; recheck vital signs and continue boluses until systolic BP > 100;
- **Treat specific Ingestions/exposures according to treatment guidelines on next pages.**
- Transport patient as early as possible.

DISRUPTED COMMUNICATIONS

In the event of a “disrupted communications” situation, the EMT-P in Solano County may utilize all portions of this treatment protocol without Base Hospital contact as is needed to stabilize an immediate patient.

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ORGANIPHOSPHATES ➤ ATROPINE 0.5 – 2.0 mg slow IVP initially. If no tachycardia or pupil dilation, use larger doses to control secretions and bronchospasm. May repeat doses of 2 - 4 mg IVP every 3 – 10 minutes as needed. ➤ TREAT SEIZURES PER PROTOCOL. Usually formulated as insecticides, these substances cause cholinergic crises characterized by bradycardia, increased salivation, lacrimation and sweating, muscle fasciculations, abdominal cramping, pinpoint pupils, incoherence or coma.	CAUSTICS & CORROSIVES ➤ Do NOT induce vomiting; ➤ Contact Base hospital or Poison Control Center and consider milk or water. EXAMPLES: <table border="0"> <tr> <td style="vertical-align: top;"> ALKALI (Bases) - Sodium Hydroxide - Drain Cleaners (Drano) - Clinitest tablets - Potassium Hydroxide (KOH) - Ammonium hydroxide (fertilizer) - Lithium Hydroxide (photo, chemicals, batteries) - Calcium Hydroxide (lime) </td><td style="vertical-align: top;"> ACIDS - Sulfuric Acid - Hydrochloric Acid OXIDIZERS - Bleach - Potassium Permanganate </td></tr> </table> Ingestion of these substances often causes intra-oral burns, painful swallowing or inability to handle secretions.	ALKALI (Bases) - Sodium Hydroxide - Drain Cleaners (Drano) - Clinitest tablets - Potassium Hydroxide (KOH) - Ammonium hydroxide (fertilizer) - Lithium Hydroxide (photo, chemicals, batteries) - Calcium Hydroxide (lime)	ACIDS - Sulfuric Acid - Hydrochloric Acid OXIDIZERS - Bleach - Potassium Permanganate
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PETROLEUM DISTILLATES ➤ In general these should not be treated in the field except with supportive measures; ➤ Vomiting should NOT be induced.	PHENOTHIAZINE REACTIONS ➤ Reassurance; ➤ DIPHENHYDRAMINE 25mg IV or 50mg IM. Characterized by restlessness, muscle spasms of the neck, jaw and back, oculogyric crisis, history of ingestion of phenothiazines (or “some pills a friend gave them”).		

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TRICYCLIC ANTIDEPRESSANTS

- For life-threatening dysrhythmias - hemodynamically significant supraventricular rhythms and ventricular dysrhythmias:
 - Hyperventilation if assisting ventilations or if intubated, or an advanced airway is used;
 - **SODIUM BICARBONATE** 1 meq/kg slow IVP.
- TREAT SEIZURES PER PROTOCOL.

These are substances which cause anticholinergic crisis characterized by altered mental status ("mad as a hatter"), fever ("hot as hell"), dilated pupils ("blind as a bat"), flushed skin ("red as a beet"), dry mucous membranes ("dry as a bone"). Frequently associated with respiratory depression, almost always accompanied by tachycardia. Widened QRS complexes and associated arrhythmias are generally signs of a life-threatening ingestion.

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